جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : زينب سيد ابراهيم الراعي
المحافظة : القليوبيه **الحي / المدينة :** قليوب **العنوان :** قليوب
رقم البطاقة : 28104011408261 **رقم التليفون :** 01554906572 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 01-04-1981 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: زينب سيد ابراهيم الراعي
National ID: 28104011408261

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستاف روسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

السرطان 57357 صحة مصر

05/20/2026

MDT FORM

Name:	Zeinab El-Sayed Ibrehem	MRN:	2859
Age:	45 Y	ID:	28104011408261
Menopausal Sta	pre	Phone:	01278720117
NRS SCORE:		Marital Status:	married
BMI:		Family History:	+ve
CO Morbidity:	free	Reason Of MDT:	post op

Clinical Data:

BX: IDC G III

ER: 0/8 PR: 3/8 HER2: 0 KI67: 70

Sonomammography:

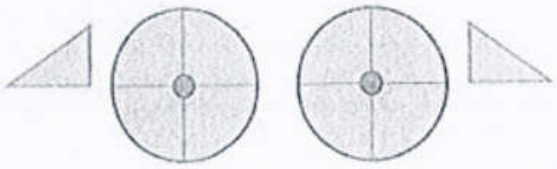
Mets Workup:

History:
initially T3 N indeterminate
BRCA negative

Recommendations:
Post LT WLE + SLNB post chemo
residual viable IDC with yy medullary like features G3
free margins
free SLNB 0/5
ypT2 ypN0 (sln)
for possibility of Adj RT
omnia essavy

MDT Outcome & Plan
PORT
CAPECITABINE X 6 MONTH
FU CTCAP/ 4 MONTHS INITIAL 2 YEARS

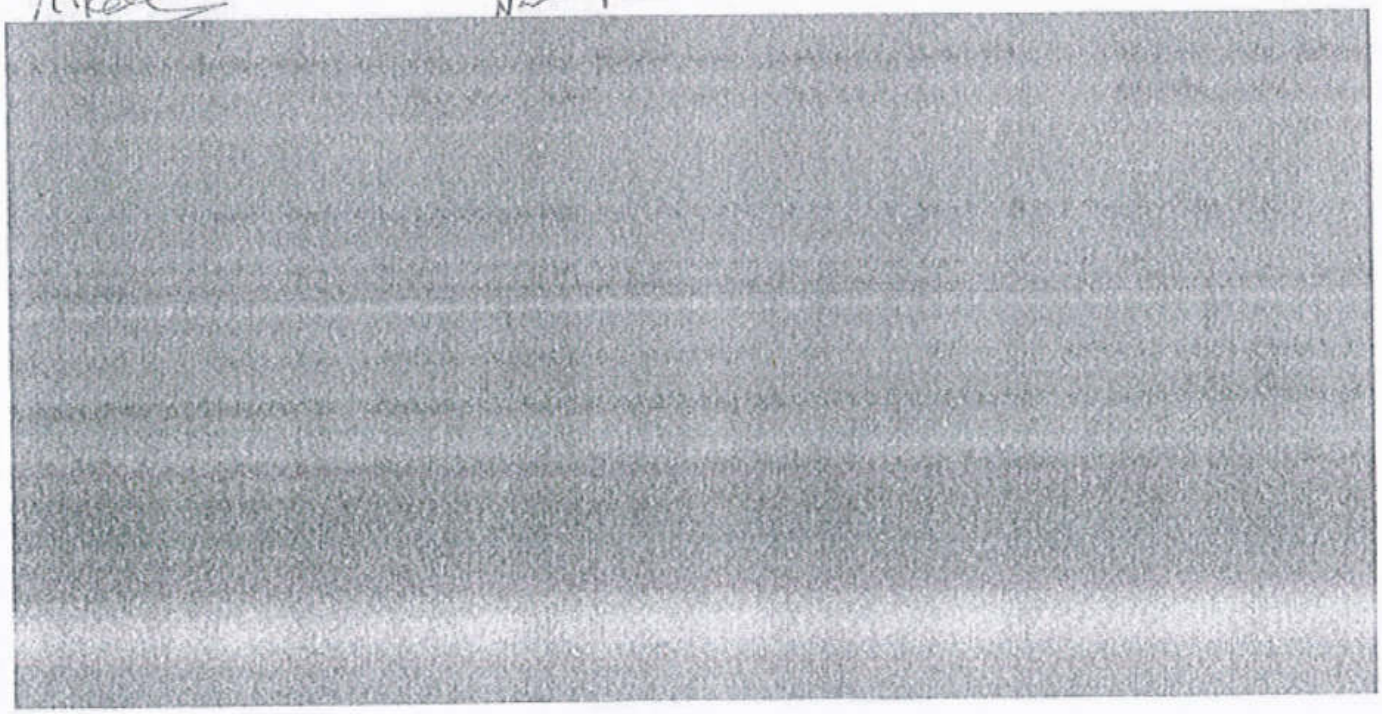
R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ محمد هيكل		د/ نرمين رمضان			



Name: زينب سيد ابراهيم **MR. :** 2859	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		**رقم طبي:**	

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient has premenopausal right-sided breast cancer, clinical stage T3 N1 M0, triple-negative phenotype. Clips were inserted, and the patient received a regimen of AC for 4 cycles followed by 8 weeks of TAXOL and carboplatin. The patient is neutropenic and is receiving Neupogen. The patient has completed 8 weeks of the TAXOL and carboplatin regimen.

The patient has left-sided triple-negative breast cancer after neoadjuvant chemotherapy, initially staged as T3 N x M0 with a partial response. On examination there is upper central distortion with no palpable lymphadenopathy.

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	
الطبيب المعالج :	dina makram		

يعتمد ,

PMSC109

جوستافروسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنت المصرية هي صحة مصر

MDT FORM 18/3/2026 G

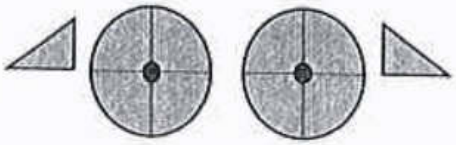
Name	زينب سيد ابراهيم	**MRN**	906/25
		ID	
Age	43	**Phone**	01554906572
Menopausal status	pre	**Marital status**	
NRS SCORE		**Family history**	
BMI		**REASON OF MDT**	Post NACT
CO morbidity	Fem.		

Clinical Data

BX: IDC GIII+

ER:	PR:	HER 2:	Ki67:
018	318	0	70%

R L



Sonomammography:

pt. sent without workup.

Mets Workup: post NACT with dose to neutropenia

interrupted cycles

TNM

Bx pt. was

~~Post HT~~ Sonomam

Recommendations: Residual malignant mass at between 12-1:00

For possibility of wire localization of Both masses then WLE+SLN Omn. Cs 11/2/26

Last cycle 25/1/2026

MDT OUTCOME & PLAN For Lt localization of Both masses. Then Lt WLE + SLN +/- LiCAP

SIGNATURE					
MEDICAL ONCOLOGY	**SURGERY**	**RADIOLOGY**	**PATHOLOGY**	**Nutrition**	**Psychiatric**
	18/3/2026				

MDT Form
(#DOCTYPE MDT_FORM)

Patient Name	:الأستاذه / زينب سيد ابراهيم الراعي	Request No.	:13008145
Patient No.	:28104011408261	Request Date	:08/03/2025
Path No.	:957/25	Verification Date	:20/03/2025
Sex	:Female	Financial Category	:اورام دار السلام - مباشره
Age	:43 Y	Department	:Out Patient
Referreing Doc	:	Registered by	:Mrs.Mayada Hegazy

Pathology Report

Clinical Data
Left breast mass

Nature of specimen
Tru cut biopsy, collected at 8/3/2025, 10:30 am

Gross examination
Multiple grayish white tissue cores up to 1.3 cm, totally submitted.
IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination
* Histological type: Invasive duct carcinoma
* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade III
* DCIS: No
* Angioinvasion: Couldn't be detected in the received biopsy
*Perineural invasion: No
• IPT (Positive control showed appropriate reactivity) :
* ER: No nuclear reaction in tumor cells (Allred score 0/8)
* PR: Weak nuclear reaction in 3 %tumor cells (Allred score 3/8)
* HER2: No membrane reaction in of tumor cells (score 0)
* Ki 67:70 % nuclear reaction of tumor cells

Diagnosis
Left breast mass, Tru cut biopsy:

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / زينب سيد ابراهيم الراعي	Request No.	: 13008145
Patient No.	: 28104011408261	Request Date	: 08/03/2025
Path No.	: 957/25	Verification Date	: 20/03/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 43 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Invasive duct carcinoma, Grade III
IPT:
* ER: Negative (score 0/8).
* PR: Positive (score 3/8).
* HER2: Negative (score 0).
* Ki67: Positive in 70% of tumor cells.

Signature
Dr. Rana Fathy
Dr.Heba Hasan

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Patient Name: Zainab Sayed Ibrahim	Date : 03/05/2025
Exam : C.T CHEST WITH CONTRAST	Age : 44 Year(s)
Medical ID: 906/2025	Pat. ID: 99277

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- **Reported case of left breast cancer.**

I Chest:

- Left breast UOQ irregular mass lesion measuring 4 X 5 cm.
- Enlarged left axillary level I and II lymph node the largest measures 9 X 19 mm.
- Prominent tiny subcentimeteric left supraclaviculari node measuring 5 mm.
- Few bilateral fissural based pulmonary nodules reaching 4 mm.
- No CT evidence of pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- Average sized liver showing homogenous parenchymal attenaution. No sizable focal lesions.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, kidneys, suprarenal glands, aorta and IVC.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- Right adnexal cystic lesion for US, it measures 6 X 4.8 cm.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- No sizable worrisome osseous lesions.

Opinion:

Reported case of left breast cancer revealed:

- **Bilateral pulmonary nodules for follow up.**
- **No sizable hepatic metastatic deposits.**
- **Right adnexal cystic lesion for sonographic correlation.**

Dr. Mohamed Mohsen, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

[[DOCTYPE:OTHER:NON]]
NON

alfa
L A B S

Collection Date: 28/06/2025
Review Date: 13/08/2025

Patient Name: الاستاذة/ زينب سيد ابراهيم الراعي
Patient No.: 176-17018
Accession No.: 17625064303
Age: 44Y

NEXT GENERATION SEQUENCING (NGS)

BRCA Report

Clinical information

Zeinab Sayed is a Cancer patient, tested for germline mutations in BRCA 1,2 for the possible applicability for targeted therapy.

Test information

Extracted DNA using QIAamp DNA Mini kit from the patient's blood sample had successfully passed appropriate libraries according to the AmoyDx BRCA Pro Panel and sequenced on the MiSeq platform (Illumina). The minAmp (minimum depth) was not less than X200.

- The coding regions of BRCA1& BRCA2 genes have been investigated using NGS technology.
- BRCA1 & BRCA2 genes have been implicated in hereditary breast, ovarian, prostate and pancreatic cancer predisposition syndrome.
- For patients with BRCA1/2 mutations (actionable pathogenic or Likely pathogenic Variants), The PARP inhibitors show clear benefit (Gradishar et al., 2021)
- All identified variants are evaluated with respect to their pathogenicity and causality, and were categorized into classes (1 – 5) as recommended by ACMG guidelines, where Class (1) Pathogenic, Class (2) Likely pathogenic, Class (3) Variant of Uncertain Significance (VUS), Class (4) Likely benign, Class (5) Benign (Richards S, et al., 2015).
-Our variant classification pipeline included variant interpreter by Illumina, Clinvar and Varsome (Kopanos C. et al.,2019).
-Only pathogenic and likely pathogenic variants with actionable therapeutic application are reported.
-The test results were reviewed, interpreted and reported by our scientific and medical experts.
-The classification of variants can change over the time.

Test result

No Actionable Pathogenic or Likely Pathogenic Variants Detected in BRCA1,2 genes.

No large rearrangement or Copy Number Variations (CNV) in BRCA1,2 genes.

Detected Variants

-The patient is not harboring Pathogenic or Likely Pathogenic Variants Detected in BRCA1,2 genes.

- The patient is unlikely to benefit of PARPI

Recommendations

We recommend Follow up with the referring Physician.

Please feel free to contact NGS team at Alfa laboratories (NGS@alfalabs.com).

[[DOCTYPE:OTHER:NON]]
NON

alfa
L A B S

Collection Date: 28/06/2025
Review Date: 13/08/2025

Patient Name: الاستاذة/ زينب سيد ابراهيم الراعي
Patient No.: 176-17018
Accession No.: 17625064303
Age: 64Y

NEXT GENERATION SEQUENCING (NGS)

BRCA Report

Test limitations

1. Intronic variants/variants in untranslated region, promoters and intronic variants are not assessed using this method.

2. Variants not detected by the assay that was performed may impact the phenotype.

3. This test is using reference databases obtained from occidental communities.

4. Cases where the genetic variant is not identified may be due to limitations in current medical knowledge or testing technology.

5. Genetic tests are largely dependent on the clinical history, medical findings and family pedigree, and therefore clinical accurate interpretation of test results may require knowing the true biological relationships in a family. Misinterpretation of results may occur if the information provided is inaccurate or incomplete.

6. The interpretation of test results is done in accordance with the clinical history, family history and other medical and laboratory data.

7. Inaccurate results in genetic testing are very rare as these tests are highly specific and accurate.

References:

1.Garrett etal.,: BRCA1/BRCA2 :CanVIG-UK Gene-Specific Guidance 2024 (https://www.cangene-canvaruk.org/canvig-uk-guidance).
2. Gradishar WJ, Moran MS, Abraham J, et al. NCCN Guidelines® Insights: Breast Cancer, Version 4.2021. J Natl Compr Canc Netw. 2021;19(5):484-493. Published 2021 May 1. doi:10.6004/jnccn.2021.0023
3. Kopanos C, Tsiolkas V, Kouris A, et al. VarSome: the human genomic variant search engine. Bioinformatics. 2019;35 (11):1978-1980. doi:10.1093/bioinformatics/bty897
4. Richards S, Aziz N, Bale S, et al. Standards and guidelines for the interpretation of sequence variants: a joint consensus recommendation of the American College of Medical Genetics and Genomics and the Association for Molecular Pathology. Genet Med. 2015;17(5):405-424. doi:10.1038/gim.2015.30

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Head of the NGS Unit

Reviewed By:

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